

SEXUAL DYSFUNCTION AND GENDER DYSPHORIA

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3 General Groups

1. **Sexual dysfunctions** (desire, arousal, orgasm, pain, and dysfunction due to a medical condition).

2. **Sexual Paraphilias** (exhibitionism, fetishism, frotteurism, pedophilia, masochism, sadism, transvestic fetishism, and voyeurism).

3. **Gender Dysphoria.**

SEXUAL DYSFUNCTIONS

- ▣ Disturbance in the processes of the sexual response cycle
- ▣ Pain associated with sexual intercourse

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- caused by psychological factors alone or a combination of psychological factors and a medical condition.

Sexual desire disorders

- involve a disruption in the desire phase of the sexual response cycle.
- **1. Hypoactive sexual desire disorder** - deficiency or absence of sexual fantasies and a lack of desire for sexual activity.

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- ▣ **2. Sexual aversion disorder**—Involves aversion to and active avoidance of genital sexual contact with a sexual partner.

Signs & Symptoms

- ❑ Lack of interest in sex
- ❑ Infrequent sexual activity
- ❑ Engages in sex often only to satisfy partner

Treatment:

- ❑ Removing the cause treats the disorder

Sexual arousal disorders

- are a disruption of the excitement phase of the sexual response cycle
- **Female sexual arousal disorder** - inability to attain or to maintain lubrication, sexual excitement, until completion of the sexual activity.
- **Male erectile disorder** - persistent or recurrent inability to attain or maintain, until completion of the sexual activity

SIGNS and SYMPTOMS

■ FEMALE SEXUAL AROUSAL DISORDER

- Decreased sexual desire
- Individual or family stress or fatigue
- Misinformation about sex & sexuality
- A pattern of dysfunctional sexual response
- Conceptual problems during childhood & adolescence about sex in general, and specifically about masturbation, incest, rape, sexual fantasies, and homosexual or heterosexual practices
- Concerns about contraception & reproductive ability
- Problems in the current sexual relationship
- Poor self-esteem & body image

TREATMENTS

▣ SEXUAL AROUSAL DISORDER

- Help to feel relax and become aware of feelings about sex, and eliminate guilt and fear of rejection.
- Psychotherapy
 - ▣ Consists of free association, dream analysis, and discussion of life patterns to achieve greater sexual awareness
- Behavioral therapy
 - ▣ Correct maladaptive patterns through systemic desensitization to situations that provoke anxiety

Orgasmic disorders

- are disruptions of the orgasm phase of the sexual response cycle.
- **Female orgasmic disorders**—Persistent or recurrent delay in, or absence of, orgasm.
- **Male orgasmic disorder**—Persistent or recurrent delay in, or absence of, orgasm
- **Premature ejaculation** - recurrent onset of orgasm and ejaculation with minimal sexual stimulation before, on, or shortly after penetration.

Treatment for Premature Ejaculation

- ❑ Insight therapy
- ❑ Behavioral therapy
- ❑ Experiential sessions involving partner
- ❑ Squeeze play
- ❑ Sensate focus exercises
- ❑ Stop-and-start technique

Nursing Interventions (PME)

- ❑ 1. Ensure a therapeutic relationship
- ❑ 2. Promote self-knowledge
- ❑ 3. Increase the level of interaction
- ❑ 4. Promote participation in care
- ❑ 5. Improve coping skills
- ❑ 6. Provide referrals
- ❑ 7. Be aware that whenever possible, a primary nurse should be assigned to the patient to ensure continuity of care.
- ❑ 8. if poses a threat, institute safety precautions

TREATMENTS

■ ORGASMIC DISORDER

- Decrease or eliminate involuntary inhibition of the orgasmic reflex.
- Experimental therapy
- Psychoanalysis
- Behavior modification
- Individual or marital therapy
- Sex therapy
- Self-stimulation & distraction techniques

Nursing Interventions

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Sexual pain disorders

- ❑ involve pain associated with sexual activity.
- ❑ **Dyspareunia**—Genital pain associated with sexual intercourse. Both male and female.
- ❑ **Vaginismus**—Persistent involuntary contractions of the perineal muscles surrounding the outer third of the vagina when vaginal penetration with penis, finger, tampon, or speculum is attempted.

Sexual dysfunction due to a general medical condition

- ❑ sexual dysfunction that is exclusively due to the physiological effects of a medical condition.

It can includes:

- ❑ pain with intercourse,
- ❑ hypoactive sexual desire
- ❑ erectile dysfunction
- ❑ orgasmic problems
- ❑ other problems as previously described.

Substance-induced sexual dysfunction

- ❑ Due to direct physiological effects of a substance (drug abuse, medication, or toxin). It may involve impaired arousal, impaired orgasm, or sexual pain.

Paraphilic Disorders



SEXUAL DISORDERS

- ▣ 1. PARAPHILIAS
- ▣ 2. SEXUAL DYSFUNCTIONS
- ▣ 3. SEXUAL PAIN DISORDERS
- ▣ 4. GENDER IDENTITY DISORDER

Paraphilia (perversion)

- ▣ Derived from the Greek words “para” meaning next to “philia” meaning love.
- ▣ paraphilic disorder is a paraphilia that is currently causing distress or impairment to the individual or a paraphilia whose satisfaction has entailed personal harm, or risk of harm, to others.* □ * DSM-5

I. Paraphilic Disorders

- ▣ defined as persistent and intense atypical sexual arousal patterns that are accompanied by clinically significant distress or impairment.

Behavior centered around:

- ▣ Inanimate objects (such as clothing)
- ▣ Suffering or humiliation
- ▣ Children or other non-consenting persons

DSM-5 CRITERIA FOR PARAPHILIC DISORDER

- ❑ Criterion A Specifies the qualitative nature of the paraphilia (e.g., an erotic focus on children or on exposing the genitals to strangers).
- ❑ Criterion B Specifies the negative consequences of the paraphilia (i.e., distress, impairment, or harm to others).

↓
Sadism

Characteristics of persons with a Paraphilia

- ❑ Beside normal sexual behaviour, paraphilic disorder involve aggression, victimization, and extreme one-sidedness.
- ❑ The behaviors exclude or harm others and disrupt the potential for bonding between persons.
- ❑ Usually lasts for 6 months.
- ❑ Always thinking to carry out their unusual behavior.

6 months

Characteristics of persons with a Paraphilia

- ❑ Overly obsessed that if the individual cannot get to their desired object, they get stressed.
- ❑ The individual will lose sight of other goals and concentrate on the fulfilment of their sexual desires if worse.

Characteristics of Paraphilic Disorders

To be diagnosed with a paraphilic disorder, **DSM-5** requires that people with these interests:

- Feel personal distress about their interest, not merely distress resulting from society's disapproval
- Have a sexual desire or behavior that involves another person's psychological distress, injury, or death, or a desire for sexual behaviors involving unwilling persons or persons unable to give legal consent

Etiology

— cause / origin
defence mechanism / nakasamayan

□ Unknown

- **Psychoanalytical Theory** - These conditions represent a regression to or a fixation at an earlier level of psychosexual development, resulting in a repetitive pattern of sexual behavior that is not mature in its application and expression.
- These conditions are all expressions of hostility in which sexual fantasies or unusual sexual acts become a means of obtaining revenge for a childhood trauma
- The persistent, repetitive nature of the paraphilia is caused by an inability to erase the underlying trauma completely

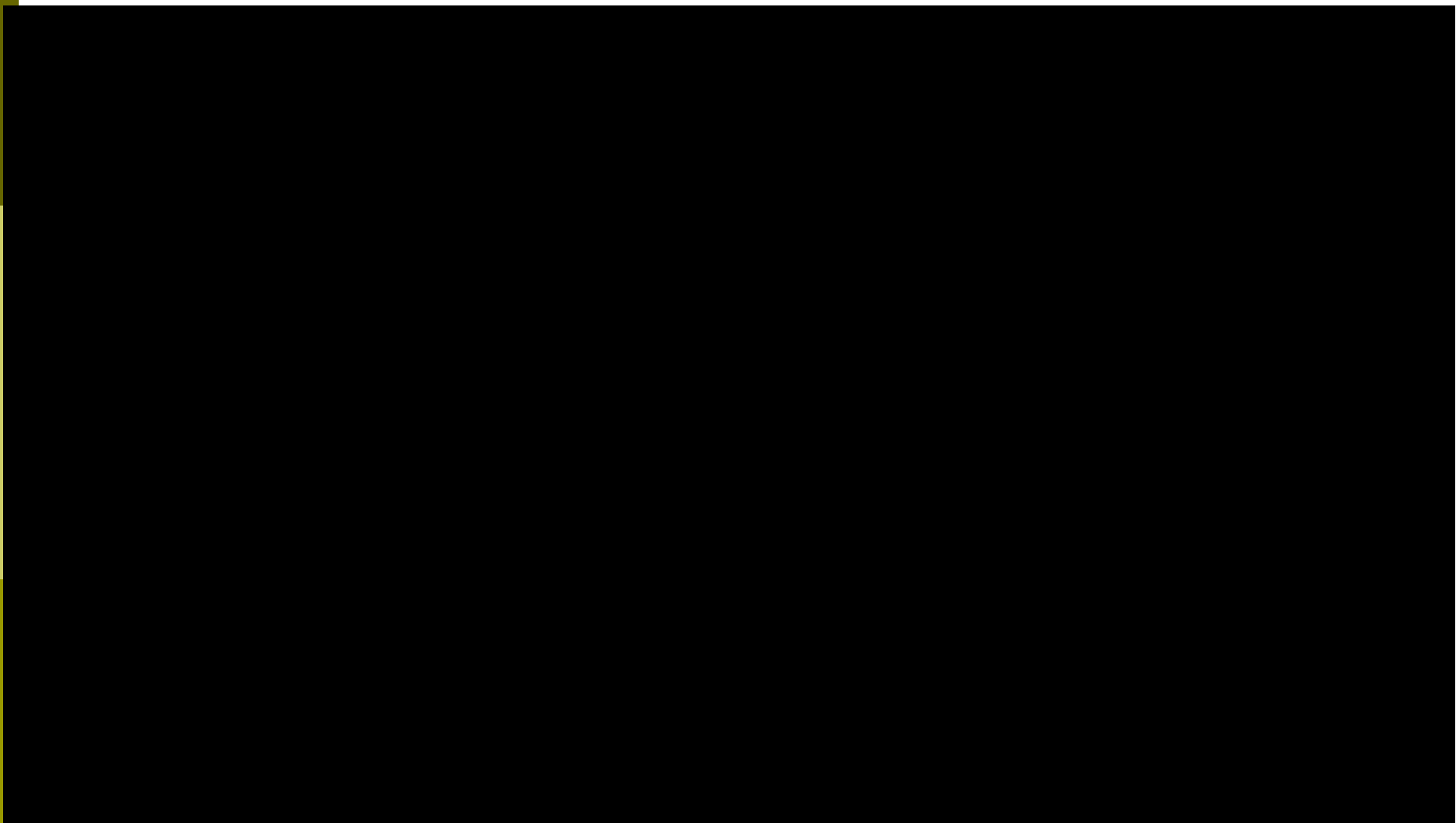
Behaviorist Theory

- The paraphilia begins via a process of conditioning.
- Nonsexual objects can become sexually arousing if they are frequently and repeatedly associated with a pleasurable sexual activity.

Biological Factors² *Scientific*

- ❑ Sexual deviance is associated with frontal and/or temporal lobe damage, Subtle defects of the right amygdala.

*small almond
limbic system*



PARAPHILIA-RELATED DISORDERS

Uncontrolled

❑ COMPULSIVE MASTURBATION

- Masturbation is the primary sexual outlet, even if the person has a stable intimate relationship

❑ PROTRACTED PROMISCUITY

one night stand

- A pattern of repeated sexual conquests in which the person can't maintain a monogamous relationship despite a desire to do so.

❑ PORNOGRAPHY DEPENDENCY

- A repetitive pattern involving the use of pornographic materials.

❑ TELEPHONE SEX

- A dependence on discussing sex over the telephone to achieve arousal.

❑ SEVERE SEXUAL DESIRE

- Person's excessive demands burden the partner & interfere with intimate relationships

❑ USE OF SEXUAL ACCESSORIES

- Marked by repetitive use of sex toys & drugs for sexual arousal

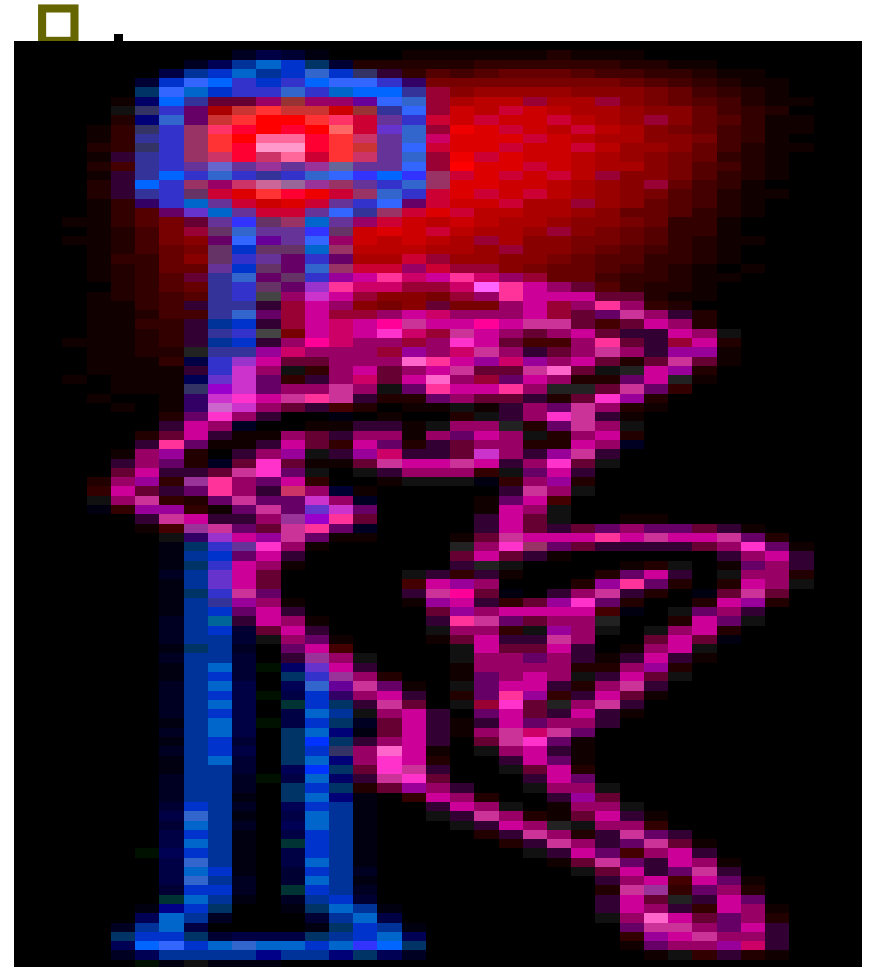
SPECIFIC PARAPHILIAS

- ❑ 1. EXHIBITIONISM
- ❑ 2. FETISHISM
- ❑ 3. TRANSVESTIC FETISHISM
- ❑ 4. PEDOPHILIA
- ❑ 5. FROTTEURISM
- ❑ 6. SEXUAL MASOCHISM
- ❑ 7. SEXUAL SADISM
- ❑ 8. VOYEURISM
- ❑ 9. RARE PARAPHILIAS

EXHIBITIONISM

- ▣ Marked by sexual fantasies, urges, or behaviors involving surprise exposure of the male genitals to strangers – primarily female passersby in public places.

The dynamic of men with exhibitionism is to assert their masculinity by showing their penises and by watching the victims' reactions -fright, surprise, and disgust.



FETISHISM

- Characterized by sexual fantasies, urges, or behaviors that involve the use of a fetish – a nonhuman object or a nonsexual part of the body – to produce or enhance sexual arousal
- Characterized by sexual fantasies, urges, or behaviors that involve the use of a fetish – a nonhuman object or a nonsexual

*for example,
shoes*

TRANVESTIC FETISHISM

- A heterosexual male dresses in female clothes (called cross-dressing) to produce or enhance sexual arousal.

male → female
dresses

(cross-dressing)

PEDOPHILIA

- Is marked by sexual fantasies, urges, or activity involving a child, usually age 13 or younger. The pedophile is erotically aroused by children and seeks sexual gratification with them.

- During sexual activity with the child, the pedophile may :
 - Undress the child
 - Encourage the child to watch him masturbate
 - Touch or fondle the child's genitals
 - Forcefully perform sexual acts on the child

FROTTEURISM

- Becomes sexually aroused from touching or rubbing against a non-consenting person. This behavior frequently occurs in crowded places, where it is easier to avoid detection.
- Common between ages 15 and 25.
- Derived from the word 'Frotter' meaning 'To rub' Frotteurism is described only in men.

bus

SEXUAL MASOCHISM

- A person gets sexual gratification from being physically or emotionally abused.
- Related disorders:
 - **INFANTALISM** – another form of sexual masochism, is a desire to be treated as a helpless infant, including wearing diapers.
 - **SEXUAL HYPOXYPHILIA** – relies on oxygen deprivation to induce sexual arousal.

DIAGNOSTIC CRITERIA

- ❑ A. Over a period of at least 6 months, recurrent and intense sexual arousal from the act of being humiliated, beaten, bound, or otherwise made to suffer, as manifested by fantasies, urges, or behaviors.
- ❑ B. The fantasies, sexual urges, or behaviors cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

SEXUAL SADISM



- A person achieves sexual gratification by inflicting pain, cruelty, or emotional abuse on others.
- The sexual sadist may verbally humiliate his partner and abuse her physically through torture, whipping, cutting, binding, beating, burning, stabbing or rape.

According to psychoanalytic theory,

- ❑ Sadism is a defense against fears of castration; persons with sexual sadism do to others what they fear will happen to them and derive pleasure from expressing their aggressive instincts.
- ❑ Contributory causes of sexual sadism: **hereditary predisposition, hormonal malfunctioning, pathological relationships, a history of sexual abuse, and the presence of other mental disorders.**

VOYEURISM

- Derives sexual pleasure from looking at sexual objects or sexually arousing situations, such as unsuspecting couple engaged in sex. He may experience an orgasm during the voyeuristic activity or later, in response to the memory of what he witnessed.

He may not witness
the act for a long time

Telephone Scatologia



- Sexual gratification is obtained from or during lewdness on the phone.

RARE PARAPHILIAS

□ COPROPHILIA

- Sexual attraction to feces

□ EMETROPHILIA

- Sexual attraction to vomit

□ HYBRISTOPHILIA

- Sexual arousal by people who have committed crimes

□ KLISMAPHILIA

- Sexual pleasure from enemas

□ NECROPHILIA

- Sexual attraction to corpses

□ PLUSHOPHILIA

- Sexual attraction to stuffed toys

□ UROLAGNIA

- Sexual attraction to urine

□ ZOOPHILIA

- Sexual attraction to animals

-
- ❑ Somnophilia (sexual arousal by a person who is unconscious)
 - ❑ Urophilia (deriving sexual pleasure from seeing or thinking about urine or urinating)

CAUSES

- ❑ 1. Unknown
- ❑ 2. Behavioral models – the child was a victim of or observer of inappropriate sexual behaviors learns to intimidate such behavior and later gains reinforcement for it.
- ❑ 3. Contributing factors – come from dysfunctional families marked by isolation and sexual, emotional, or physical abuse.

SIGNS and SYMPTOMS

- ❑ Anxiety
- ❑ Depression
- ❑ Development of a hobby or an occupation change that makes the paraphilia more accessible
- ❑ Disturbance in body image
- ❑ Guilt or shame
- ❑ Ineffective coping
- ❑ Multiple paraphilia at the same time
- ❑ Purchase of books, videos or magazines related to paraphilia
- ❑ Recurrent fantasies about paraphilias
- ❑ Sexual dysfunction
- ❑ Social isolation
- ❑ Troubled social or sexual relationships

Treatments

- ❑ 1. Behavioral therapy – aversive stimuli, such as bad odors or electric shocks.
- ❑ 2. Social skills training
- ❑ 3. Sex offender program
- ❑ 4. group therapy
- ❑ 5. rational thinking group
- ❑ 6. structured sexual disorder process group
- ❑ 7. Educational sessions
- ❑ 8. therapeutically structured recreational activities – adventure based programming, arts and crafts, team sports, and experimental games
- ❑ 9. Resident and parent participation in treatment reviews
- ❑ 10. alcohol and drug awareness programs
- ❑ 11. values clarification
- ❑ 12. Independent living skills
- ❑ 13. Vocational exploration
- ❑ 14. Pharmacologic treatment – hormones if with compulsive thinking

Nursing Interventions

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IV. GENDER IDENTITY DISORDER

- is a condition with which a person who has been assigned one gender(usually at birth on the basis of their sex, but compare intersexuality), but identifies as belonging to another gender, or does not conform with the gender role their respective society prescribes to them.
- It is a psychiatric term for what is widely known by terms like **transexuality, transgender and transvestism or cross dressing**

SIGNS and SYMPTOMS

- ❑ Believed they were born the WRONG SEX
- ❑ Anxiety
- ❑ Attempts to mask or remove sex organs
- ❑ Cross dressing
- ❑ Depression
- ❑ Disturbance in body image
- ❑ Dreams of cross-gender identification
- ❑ Fear of abandonment by family & friends
- ❑ Finding one's genitals "DISGUSTING"
- ❑ Ineffective coping strategies
- ❑ Peer ostracism
- ❑ Preoccupation with appearance
- ❑ Self-hatred
- ❑ Self-medication with hormonal therapy
- ❑ Strong attraction to stereotypical activities of the opposite sex
- ❑ Suicide attempts or ideation

TREATMENTS

- ❑ Individual and couples therapy
- ❑ Hospitalization – if with potential for violence
- ❑ Group or individual psychotherapy
- ❑ Family therapy – for a child, role modeling purposes
- ❑ Sex re-assignment surgery – through hormonal therapy and sex-change surgery, optional, for concomitant personality disorders

Nursing Interventions

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SEXUAL ORIENTATION

- Sexual preference for erotic partners of the same, opposite, or either sex
 - Heterosexuality
 - **Homosexuality and Lesbianism**
 - Alternative sexual behavior, applied to sexual relation between people of the same sex
 - Bisexual
 - Asexual

ETIOLOGY OF HOMOSEXUALITY

- ❑ Hereditary tendencies
- ❑ Imbalance of sex hormones
- ❑ Environmental influences and conditioning factors, related to learning and psycho-dynamic theories.
 - Defense against unsatisfying relationship with father
 - Unsatisfactory and threatening early relationship with opposite sex
 - Oedipal attachment to parent
 - Seductive parent (incest)
 - Castration fear
 - Labeling and guilt leading to sexual acting out
- ❑ Preferred choices (As in life-style)

- A man is having considerable distress
about his wife's inability to enjoy sexual relations after the birth of their child. He has never been unfaithful to his wife but is frustrated with their current relationship. He is considering having an affair with one of his co workers. What would be the best approach for this patient? Is referral to a sex therapist appropriate?

- During a nursing assessment, a young adult woman reveals that she is a lesbian and that, she is currently living with a partner. She has not yet told her parents about her sexual orientation and is experiencing some anxiety about sharing this part of her life with her family. Her partner is supportive and encourages the young woman to take her time in telling her family members. Is this patient experiencing symptoms of a gender identity disorder? Explain

Thank you!



yes!